

Coverage of obesity drugs in Medicaid: what it did, who it reaches, what it could cost

Wegovy and Zepbound in state Medicaid programs · October 2026

12.2

additional prescriptions per 1,000
enrollees per quarter (95% CI 8.4
to 16.0)

128.8M

U.S. adults meet the FDA label
criteria (lower bound)

\$161.3M

five-year net cost for 1 million
enrollees; \$2.69 per member per
month

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erickyegon.github.io/incretin-access-value

The four questions and the answers: who is eligible, what coverage did, who prescribes, and what it could cost

128.8 million

Who is eligible?

U.S. adults meet the FDA label criteria (lower bound);
17.8 million have Medicaid.

12.2 per 1,000

What did coverage do?

Prescriptions per 1,000 enrollees per quarter (95% CI
8.4 to 16.0), robust in all 15 estimable alternative
analyses.

55.8%

Who prescribes?

of 2024 Part D incretin claims came from primary care;
nurse practitioners and physician assistants wrote
27.7%.

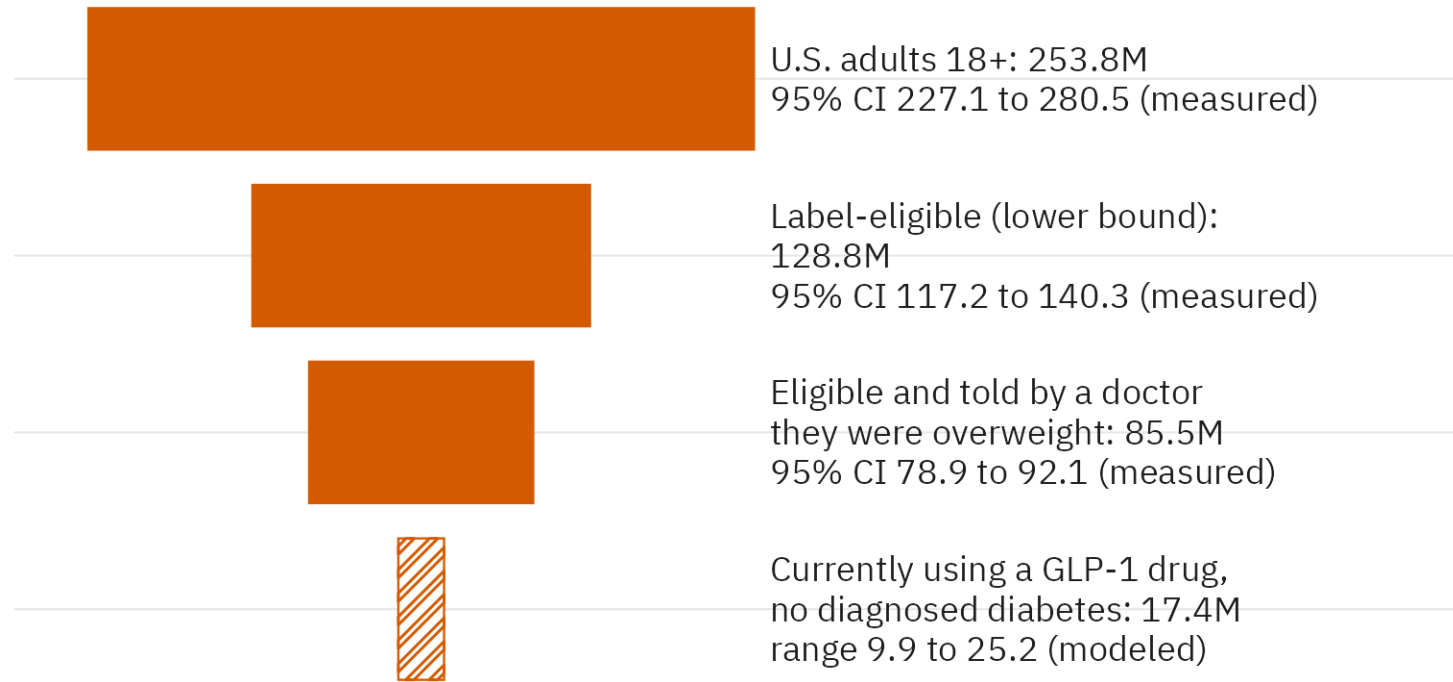
\$161.3 million

What could it cost?

net over five years for 1 million enrollees (\$2.69 per
member per month); the rebate is the largest unknown.

Sources: NHANES, CMS State Drug Utilization Data and Medicaid enrollment, Medicare Part D Prescribers, Module E budget model. Associations and scenarios, not forecasts.

Market today: about half of U.S. adults meet the label criteria, and about 30.4 million already use a GLP-1 drug



128.8M

U.S. adults meet the label criteria (95% CI 117.2 to 140.3 million), a lower bound

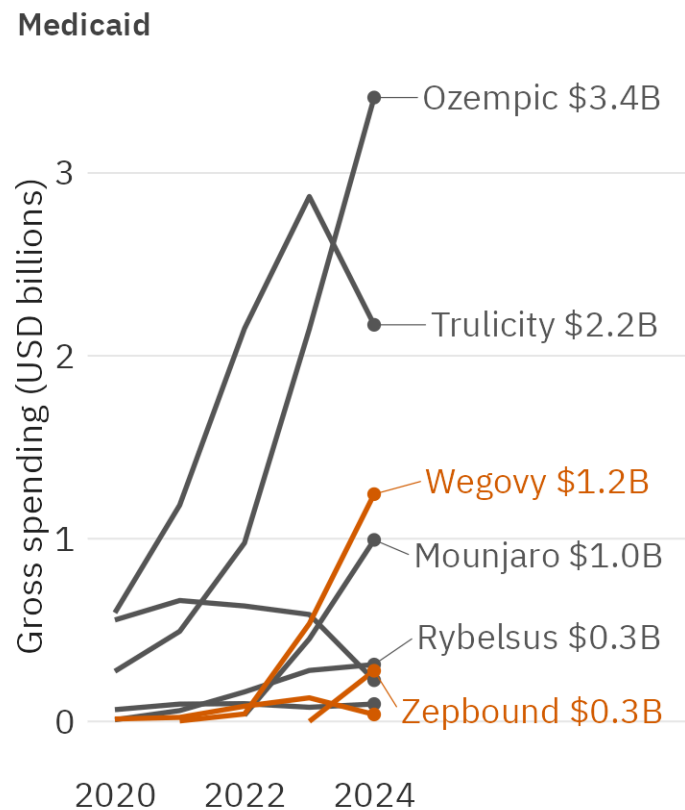
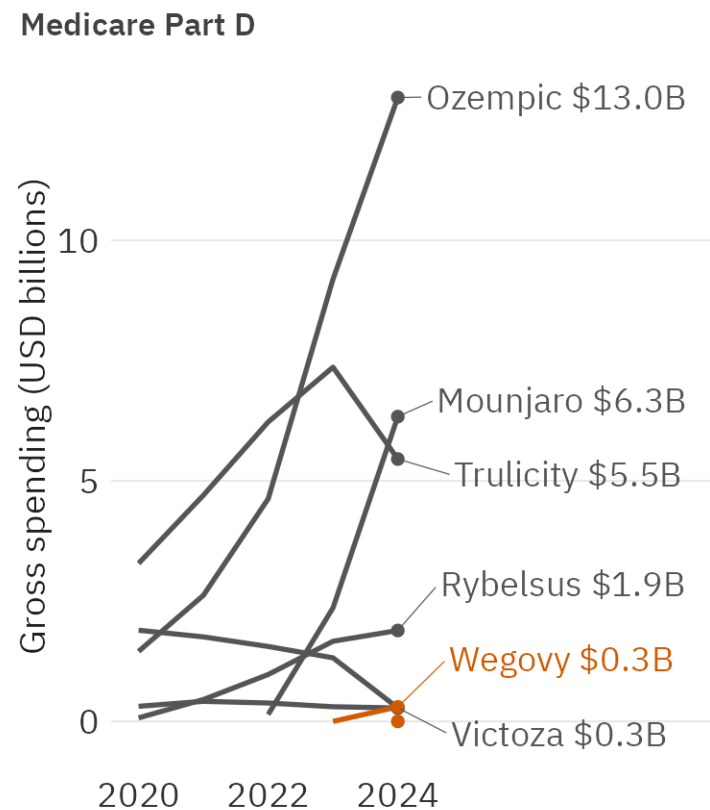
17.8 million of them have Medicaid

30.4 million adults currently use any GLP-1 drug (modeled)

Solid bars are measured; hatched bars are modeled

Source: NHANES 2021-2023 (measured) and KFF Health Tracking Poll, February 24 to March 2, 2026 (modeled).

Market today: gross spending reached \$27.5 billion in Part D and \$8.8 billion in Medicaid in 2024, before rebates



\$8.8B

gross Medicaid spending on these drugs in 2024, before rebates

Part D: \$27.5B, which includes diabetes and other covered uses
Ozempic and Mounjaro lead the growth; obesity-labeled brands in orange

Source: CMS Medicare Part D Spending by Drug and Medicaid Spending by Drug (gross of rebates).

Incretin access & value · Erick Kiprotich Yegon · erickyegon.github.io/incretin-access-value

Landscape: injectables, new oral options, a Medicare Bridge and a \$245 price are arriving while some states end coverage

Products and oral options

Wegovy (injection and tablets), Zepbound (KwikPen) and Foundayo (tablets) are covered under the Bridge.

Medicare GLP-1 Bridge

A short-term demonstration, July 1, 2026 to December 31, 2027, with a \$50 copay (CMS).

The \$245 price

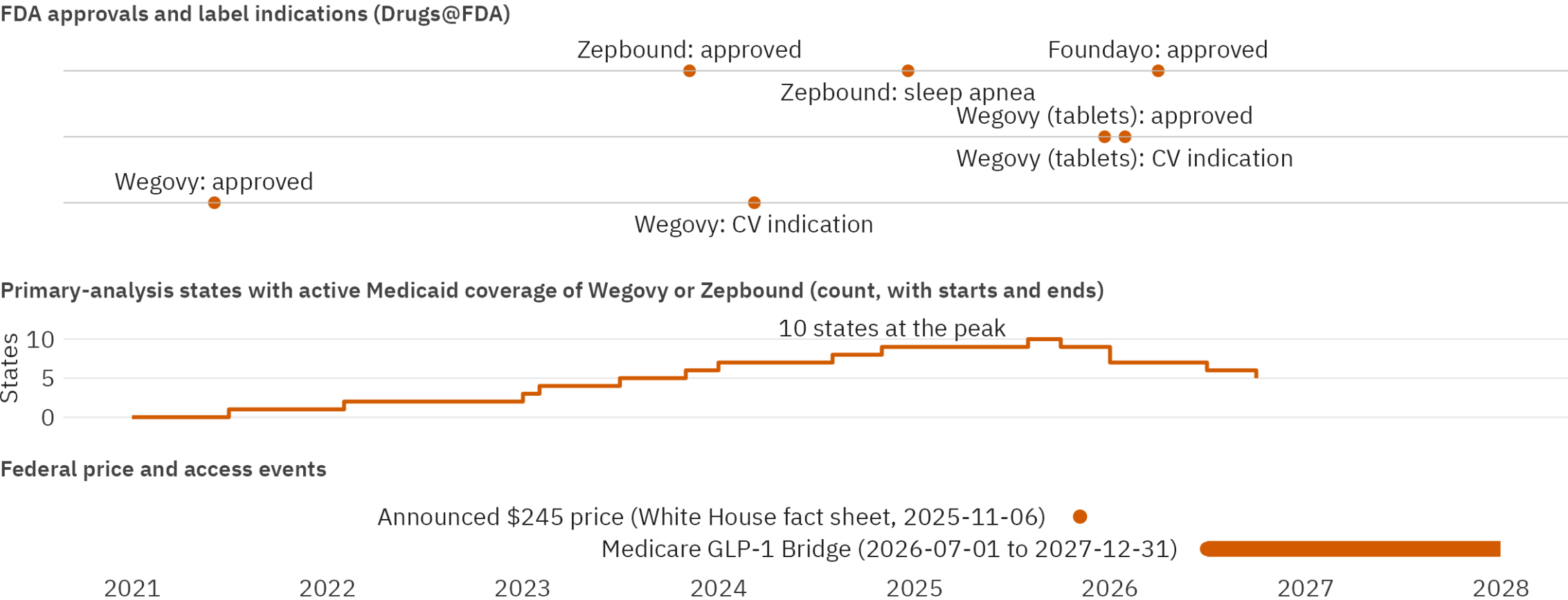
The White House (November 2025) announced a Medicare price of \$245 a month and said state Medicaid programs will have access at those prices.

State coverage is changing

Several states ended or paused coverage in late 2025 and early 2026. Pipeline: 634 registered obesity studies, 59 ongoing Phase 3.

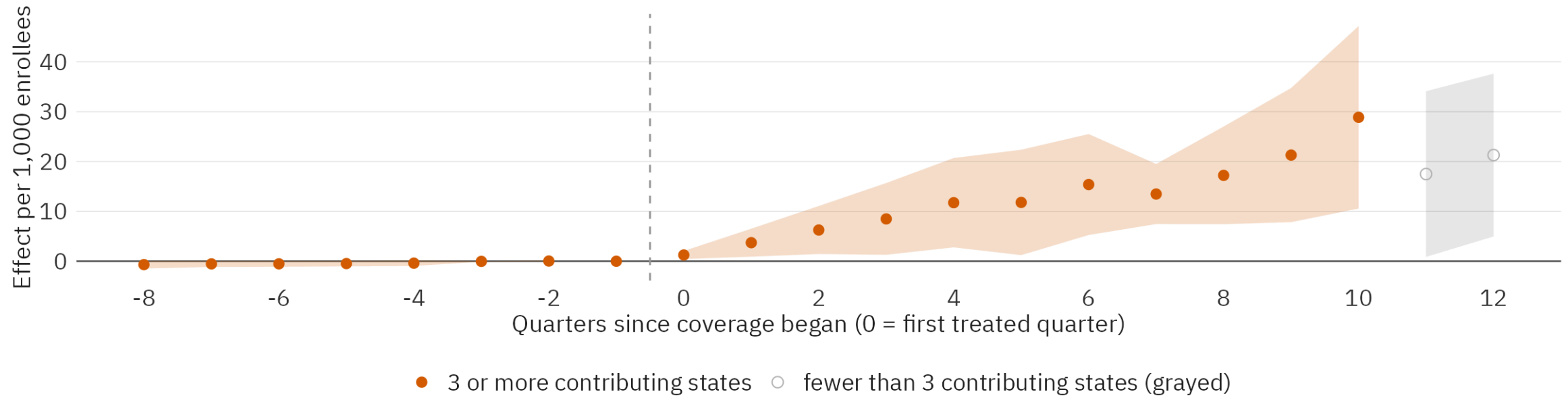
Sources: CMS Medicare GLP-1 Bridge pages; White House fact sheet, November 2025; state Medicaid documents; ClinicalTrials.gov (registry last updated 2026-10-02).

Timeline: label changes, state coverage starts and ends, the \$245 price and the Medicare Bridge cluster in 2024 to 2026



Sources: FDA Drugs@FDA labels; state Medicaid documents (primary-analysis states); White House fact sheet, November 2025; CMS Medicare GLP-1 Bridge pages.

Access: coverage was associated with an estimated 12.2 more prescriptions per 1,000 enrollees per quarter



12.2

per 1,000 enrollees per quarter, 95% CI
8.4 to 16.0

1.3 to 17.2

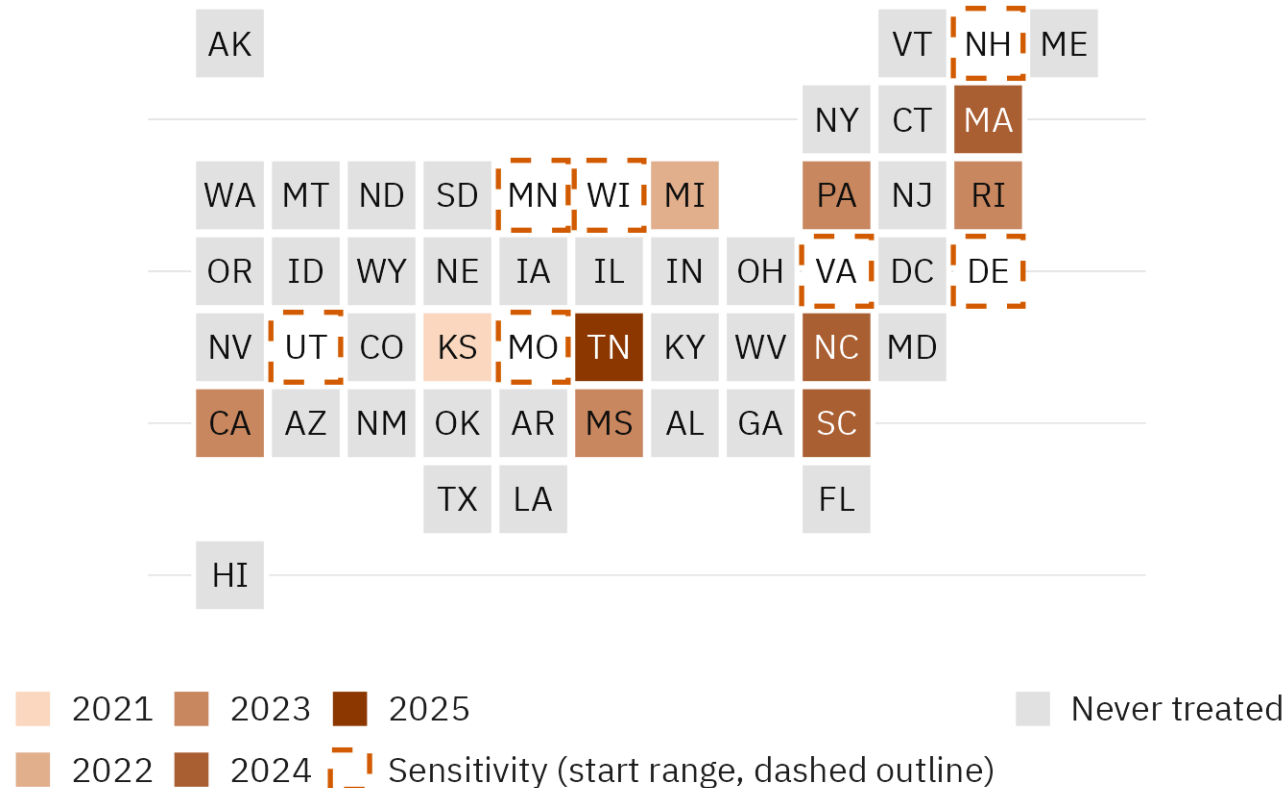
first quarter to eighth, per 1,000 (market
growth included)

0.4

placebo effect (95% CI -0.1 to 0.8): no
effect before coverage

Source: CMS SDUD and Medicaid enrollment; 10 covering and 34 never-covering jurisdictions. Callaway and Sant'Anna estimator; part of the growth is national market growth.

Access: 10 states form the primary analysis, and their prior authorization and BMI rules differ



9 of 10

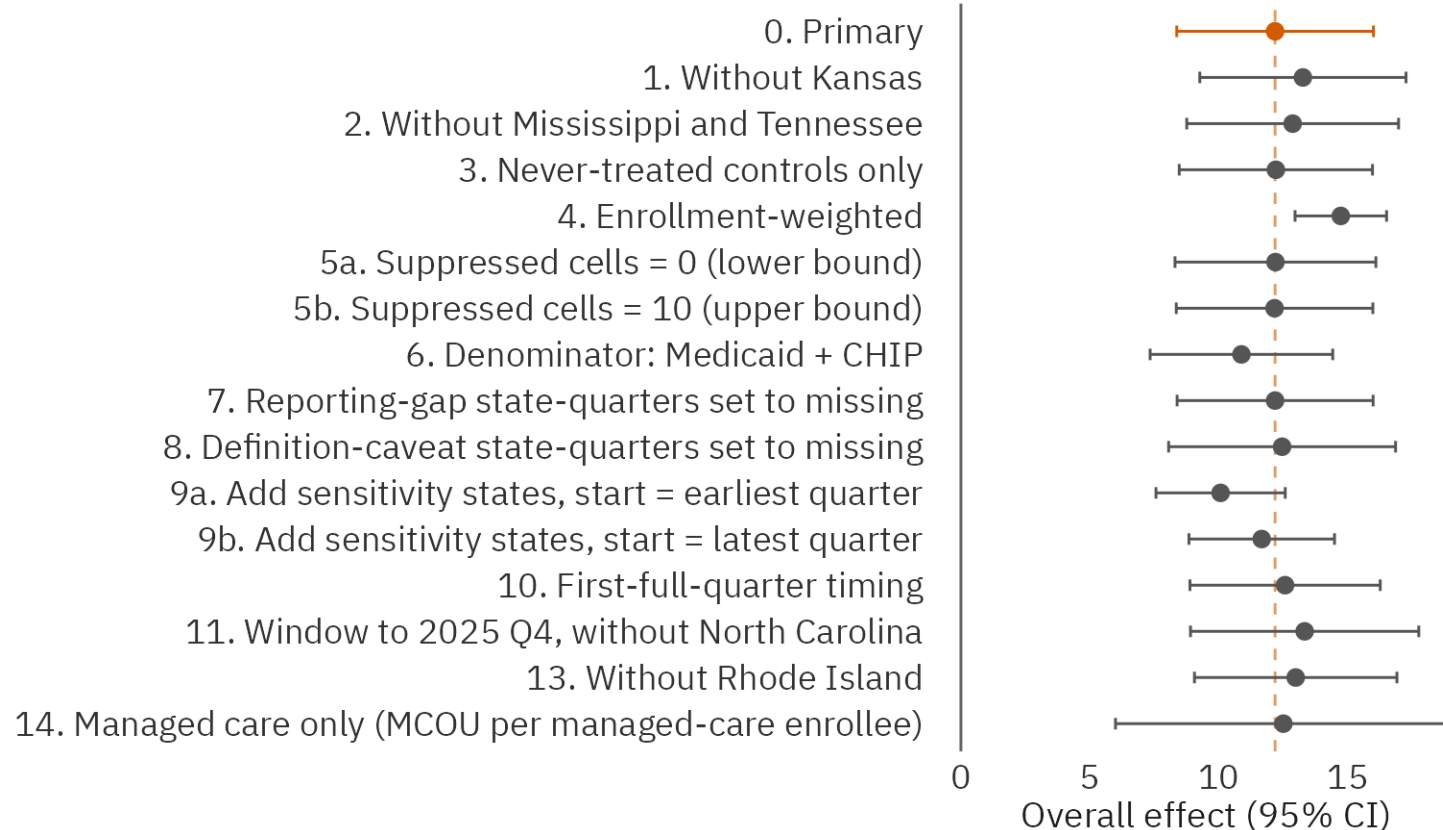
primary states have prior authorization documented (California's is not found in sourced documents)

BMI thresholds documented for 12 of 17 covering states; step therapy for 13

7 more states with uncertain start quarters appear only in a sensitivity analysis

Source: state Medicaid documents and CMS approvals (docs/sources); criteria table: coverage_um_criteria.csv in the repository.

Access, robustness: the estimate holds in all 15 estimable alternative analyses, from 10.1 to 14.7 per 1,000



All 15

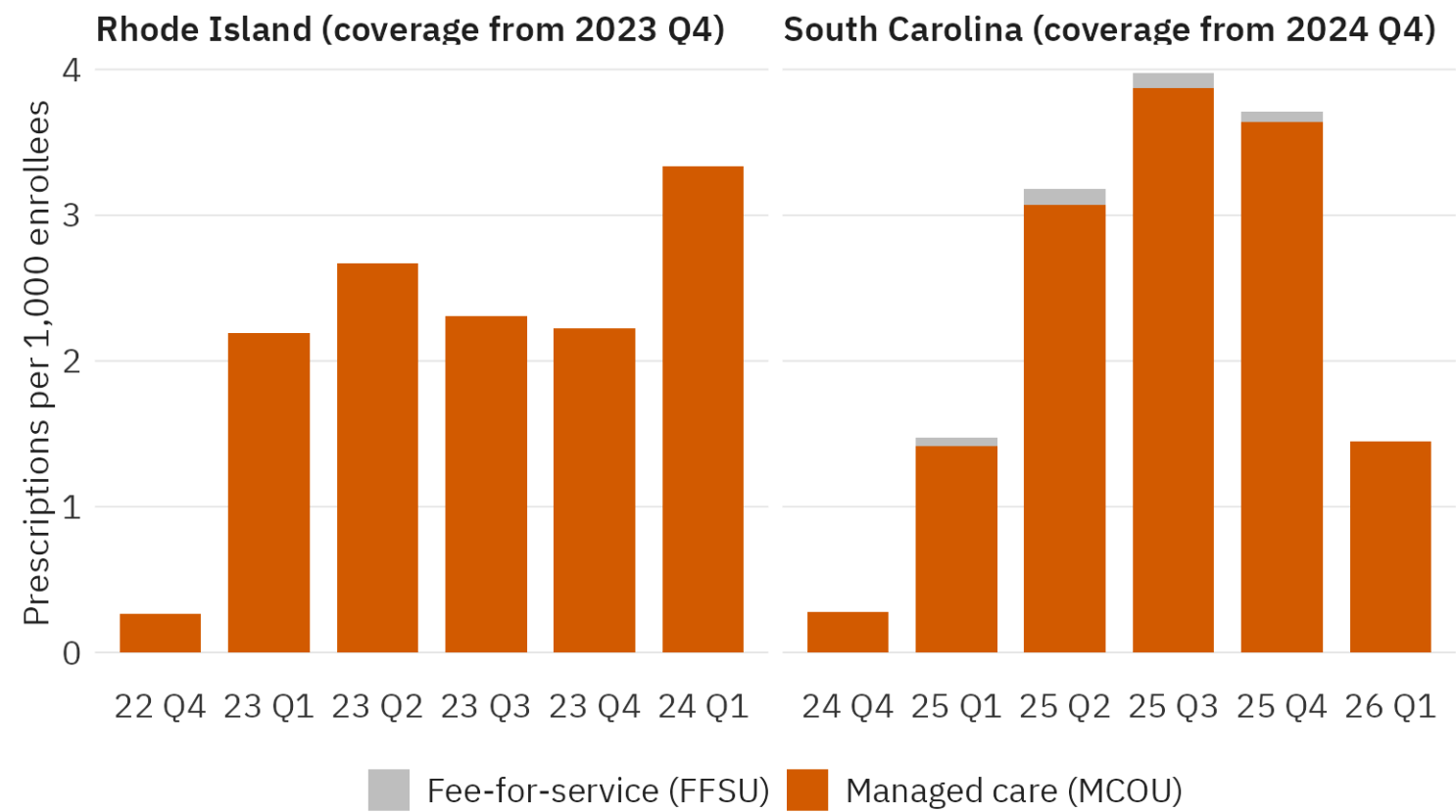
estimable alternative analyses give an interval that excludes zero

Overall effect ranges 10.1 to 14.7 per 1,000 across them

Fee-for-service only is not estimable, so it is not shown

Source: Module C specification table (95% CIs); orange = primary analysis.

Access, detail: coverage operates through managed care, so a fee-for-service-only estimate is not reliable



97.5%

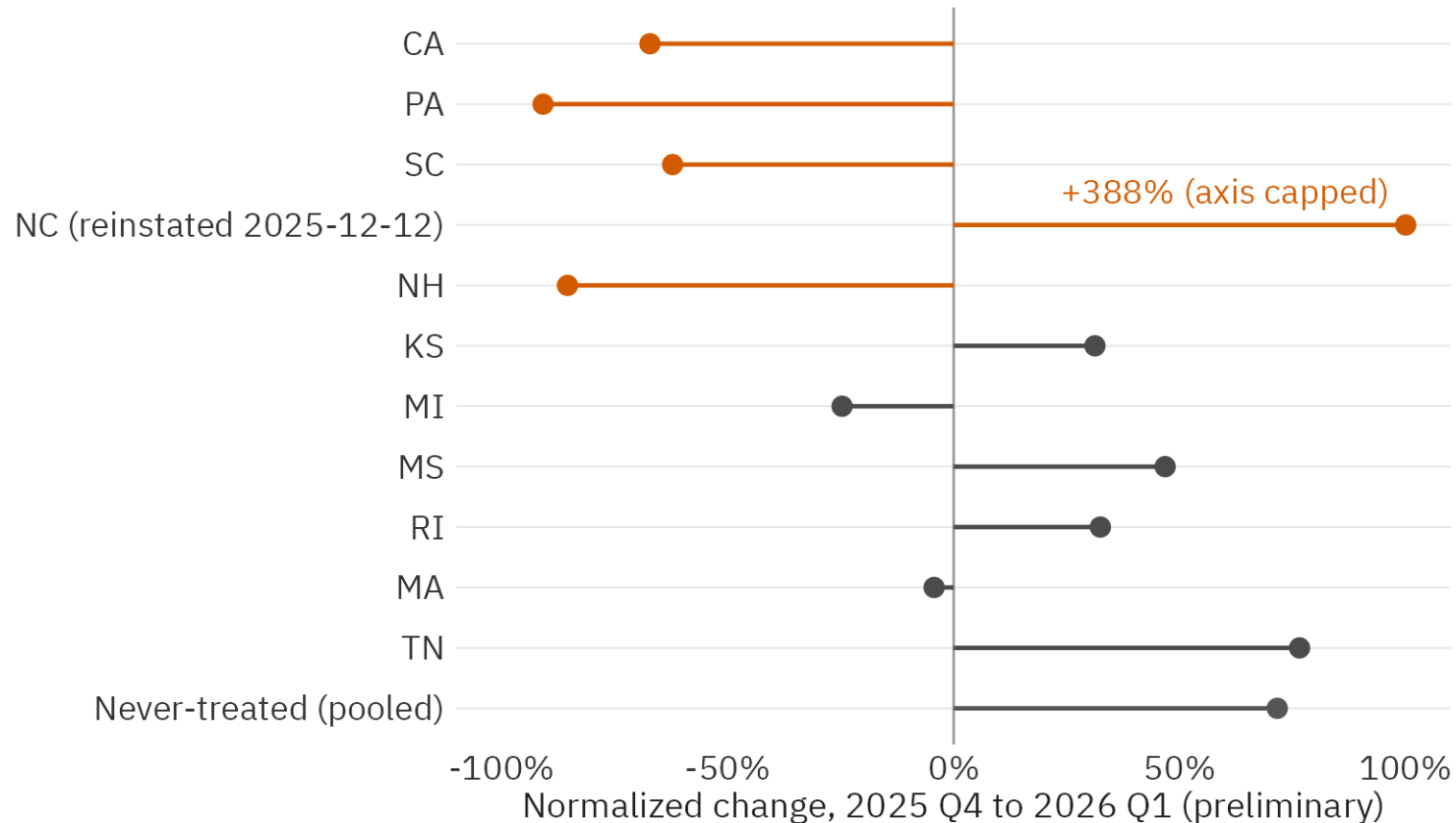
of South Carolina’s observed prescriptions came through managed care in 2025 Q3

100.0% in every Rhode Island quarter

A managed-care-only estimate is 12.5 per 1,000 managed-care enrollees (95% CI 6.0 to 19.0)

Source: CMS State Drug Utilization Data, observed (unsuppressed) prescriptions per 1,000 enrollees; gross of rebates.

Access, detail: in preliminary data, the first withdrawals appear steep and show how fast coverage can reverse



-67.2%

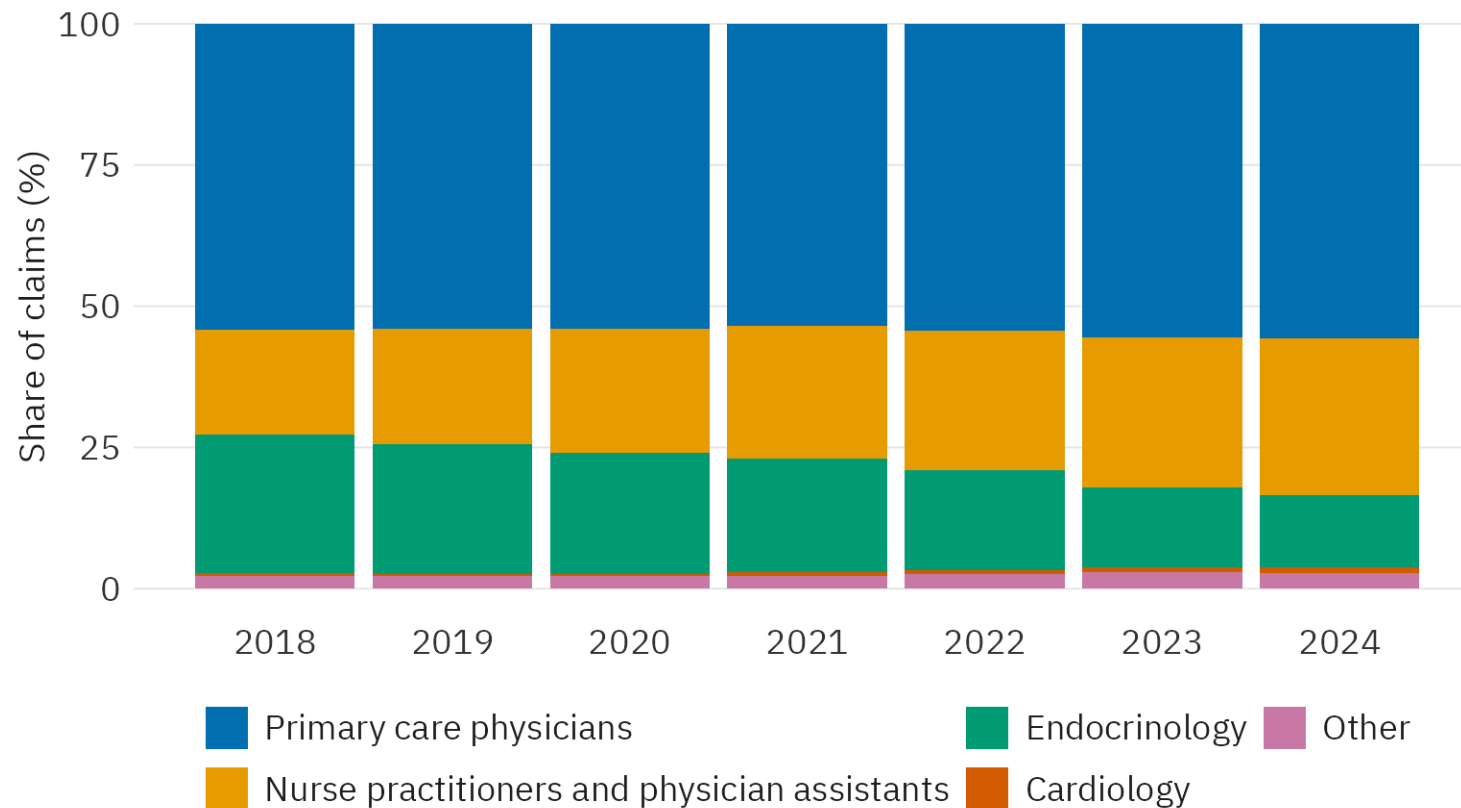
California, normalized change in prescriptions after coverage ended (preliminary)

-90.8% in Pennsylvania

2026 Q1 is a preliminary quarter; descriptive only, no model

Source: SDUD 2025 Q4 to 2026 Q1. Normalized change = obesity-labeled change relative to the state's all-drug change. North Carolina's coverage lapsed 2025-10-01 and was reinstated 2025-12-12, so its rebound is not a withdrawal.

Prescribers: primary care wrote 55.8% of 2024 Part D incretin claims, nurse practitioners and physician assistants 27.7%



55.8%

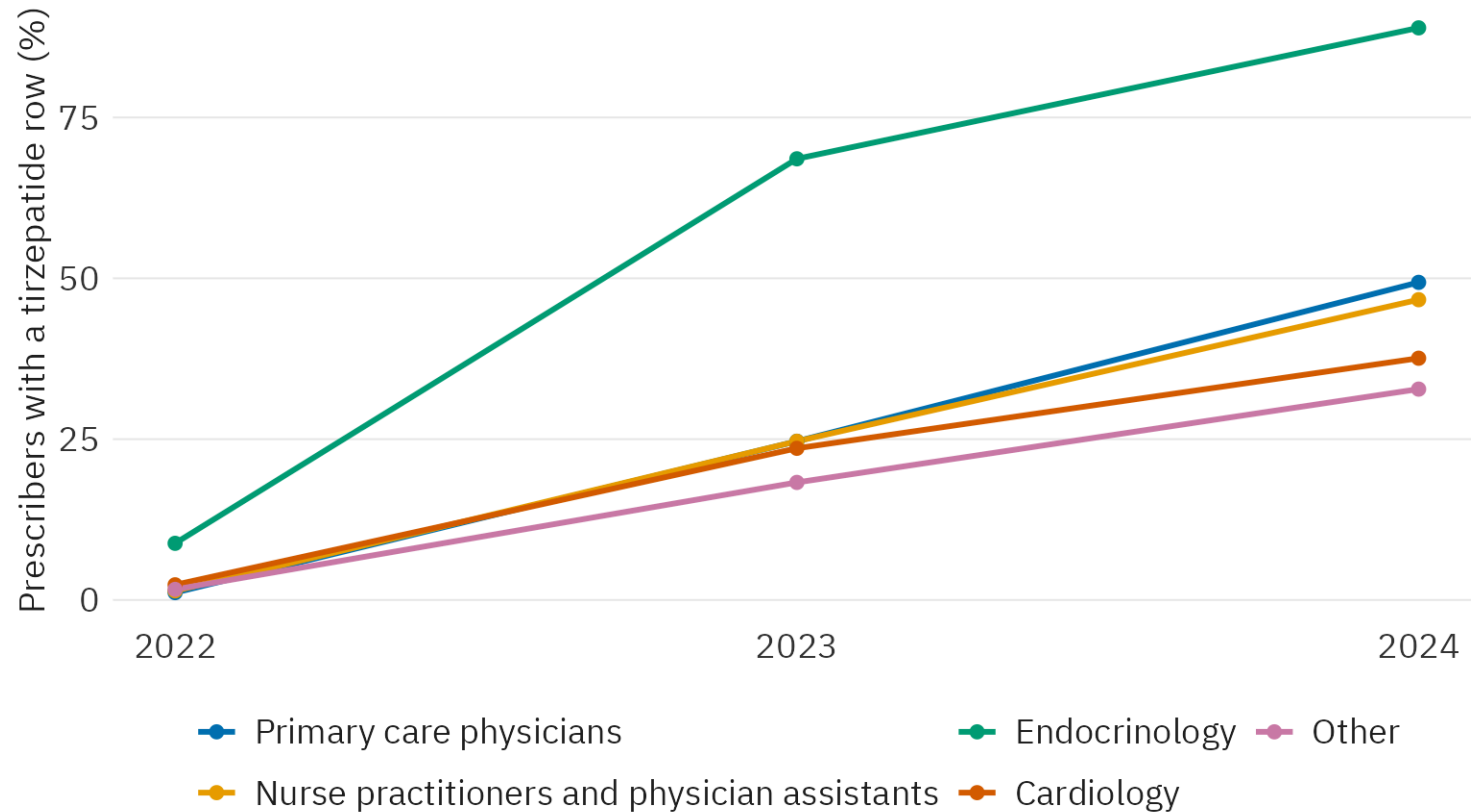
of Part D incretin claims came from primary care physicians in 2024

27.7% from nurse practitioners and physician assistants

Part D reflects diabetes and other covered uses, not obesity-brand adoption

Source: CMS Medicare Part D Prescribers (rows with at least 11 claims).

Prescribers: tirzepatide reached 48.6% of Part D incretin prescribers by 2024, from 1.6% in 2022



48.6%

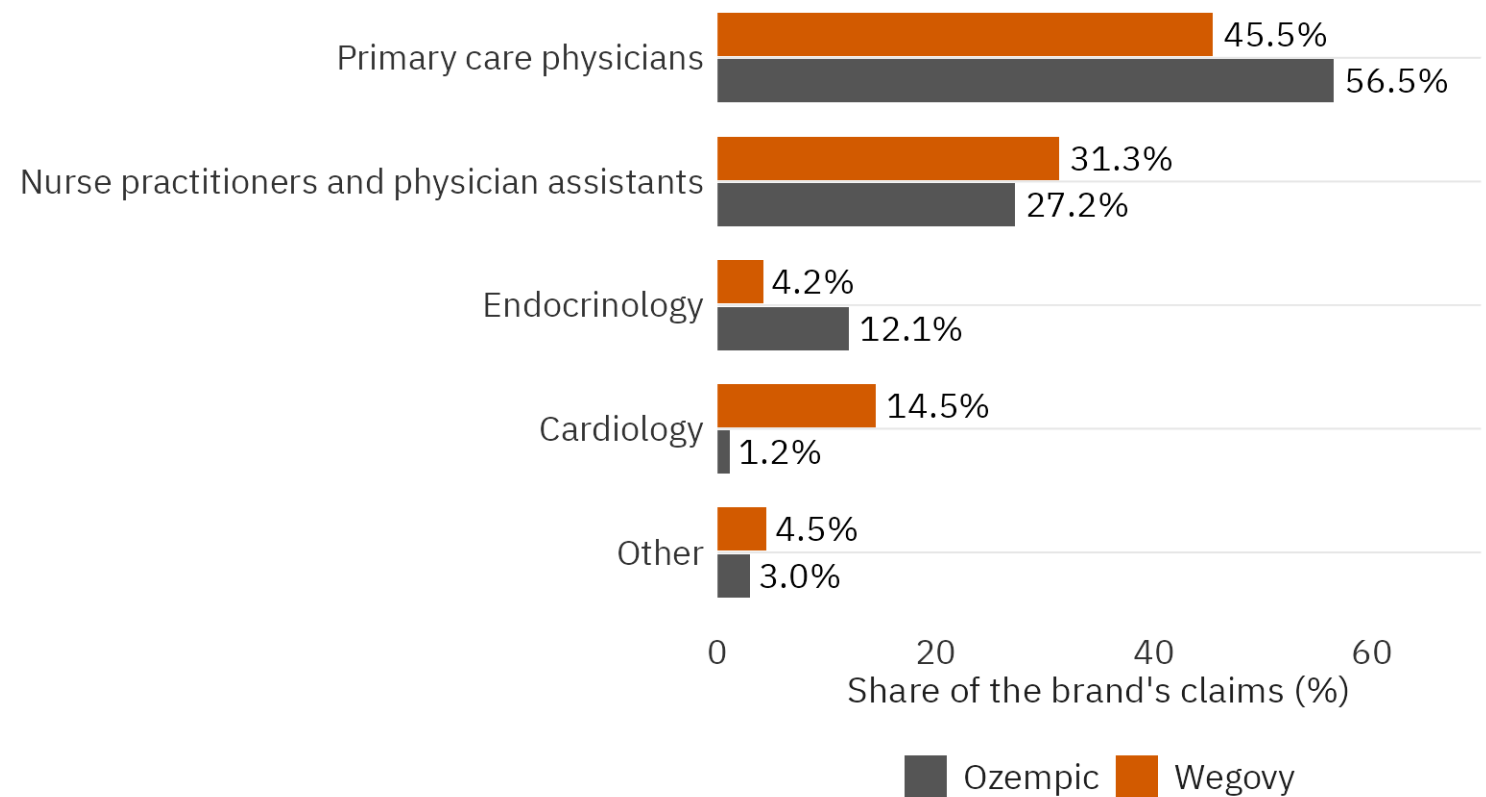
of incretin prescribers had a tirzepatide row in 2024, from 1.6% in 2022

Endocrinology leads adoption; primary care and nurse practitioners and physician assistants follow

Part D reflects diabetes and other covered uses

Source: CMS Medicare Part D Prescribers. Share of prescribers with at least one incretin row who also have a tirzepatide row.

Prescribers: cardiology wrote 14.5% of Wegovy's Part D claims, against 1.2% for Ozempic



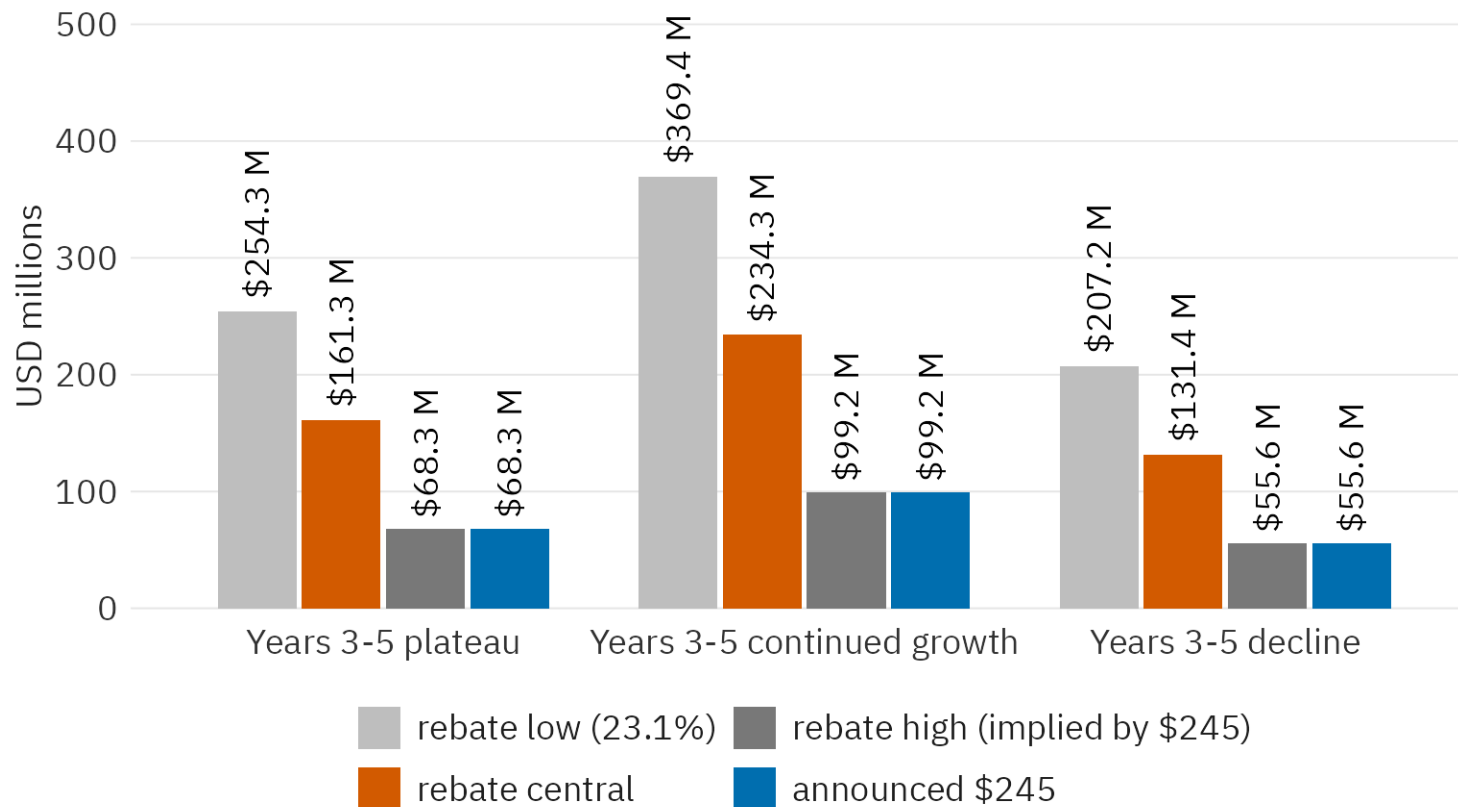
14.5%

of Wegovy's Part D claims came from cardiology in 2024, against 1.2% for Ozempic

Wegovy gained a cardiovascular indication in 2024; 76,176 Wegovy claims from 3,956 prescribers
One year of data: descriptive only

Source: CMS Medicare Part D Prescribers, 2024. Part D reflects diabetes and other covered uses, not obesity-brand adoption.

Value: the five-year net cost for 1 million enrollees is \$161.3 million centrally, within \$55.6 to \$369.4 million



\$161.3M

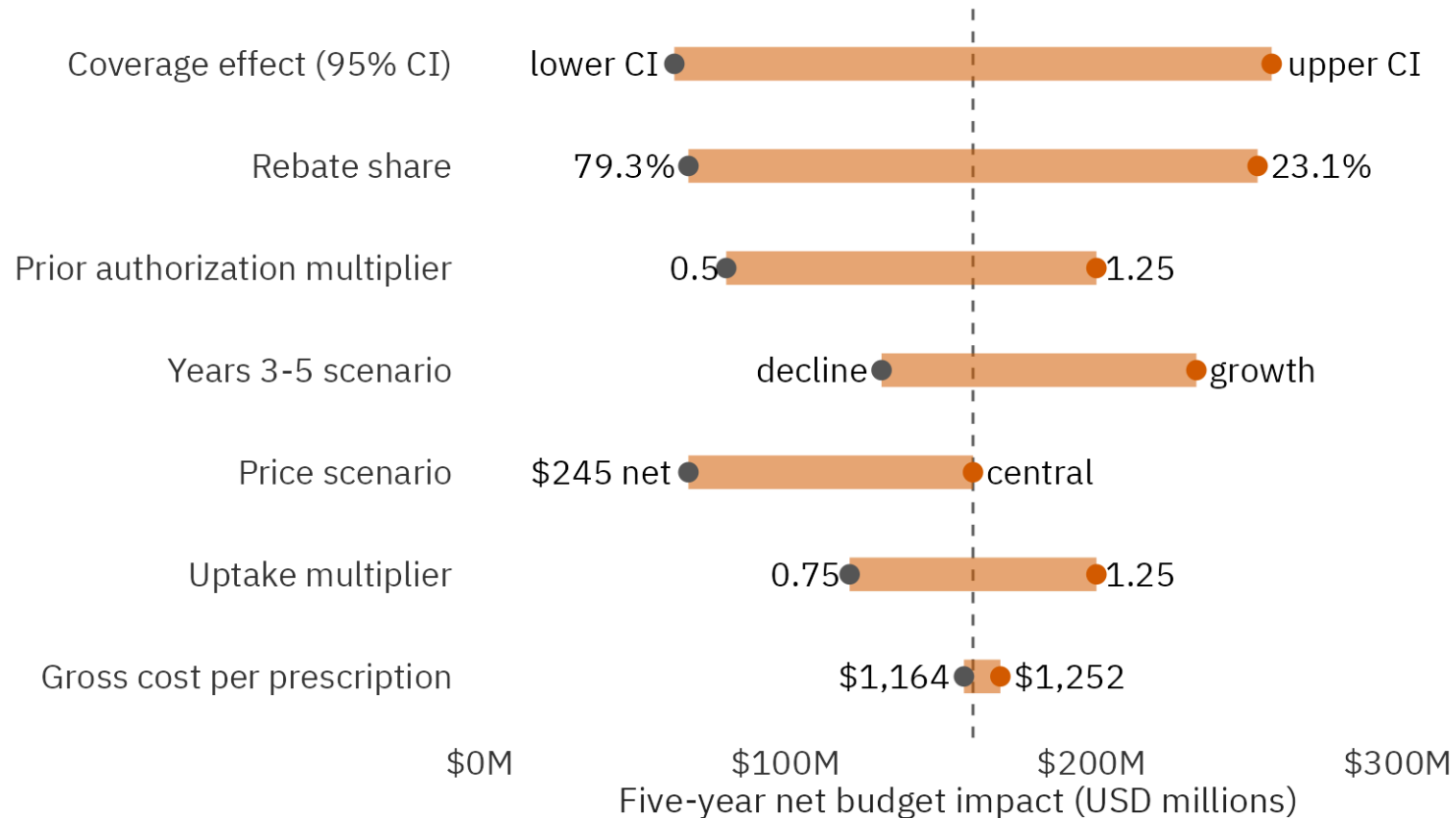
central five-year net cost (\$2.69 per member per month)

Range across scenarios: \$55.6M to \$369.4M

Rebates, prior authorization and years 3 to 5 are assumptions; no medical cost offsets

Source: Module C effects, SDUD gross reimbursement, statutory rebate floor (42 U.S.C. 1396r-8), White House fact sheet.

Value: the rebate and the uncertainty in the coverage effect move the answer most



51.2%

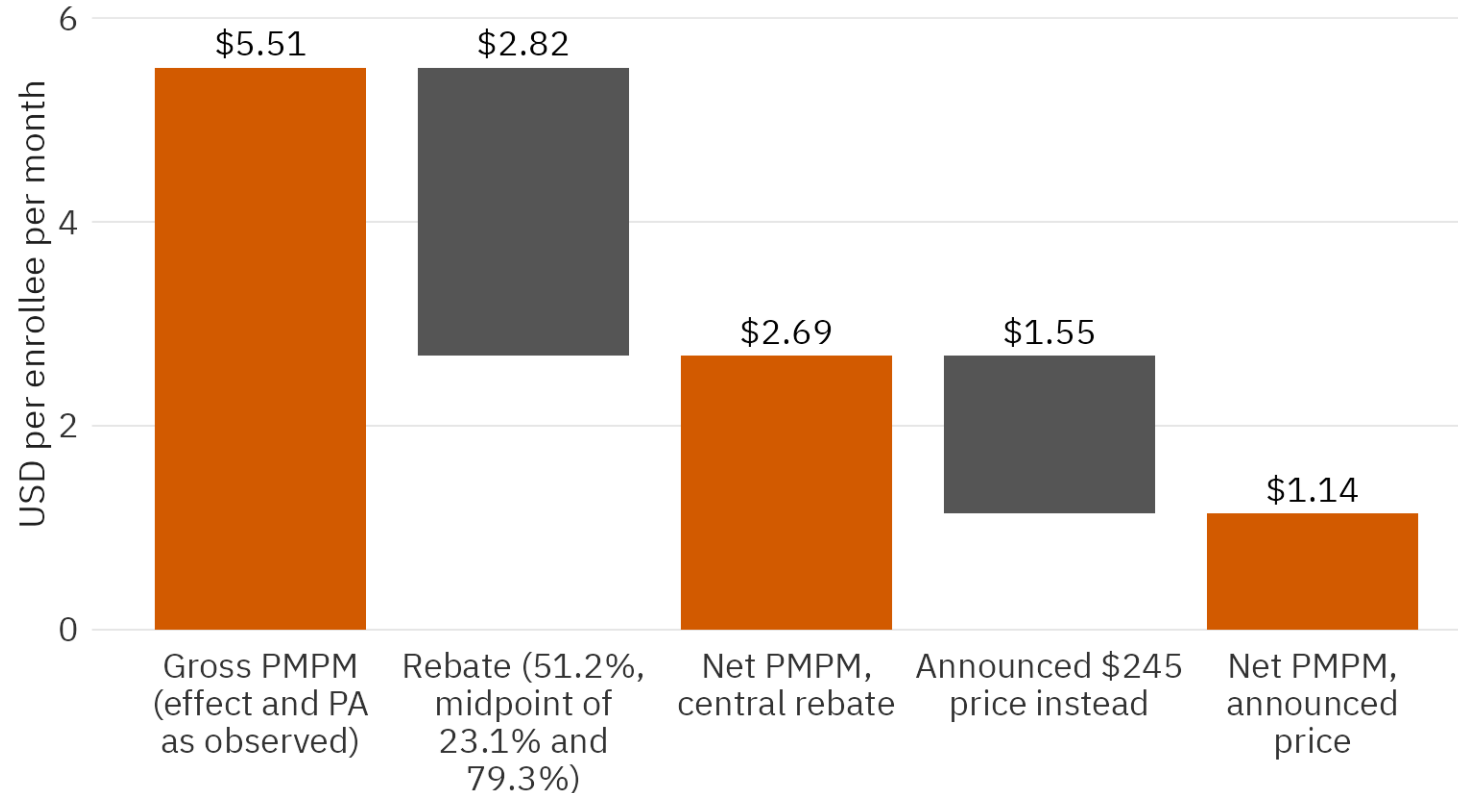
central rebate, the midpoint of 23.1% (statutory minimum) and 79.3% (implied by the \$245 price)

Actual net prices are confidential, so the rebate is an assumption

Each bar moves one input across its range with the others at central values

Source: Module E one-way sensitivity.

Value: per enrollee per month, a gross \$5.51 becomes \$2.69 net at the central rebate and \$1.14 at the announced price



\$2.69

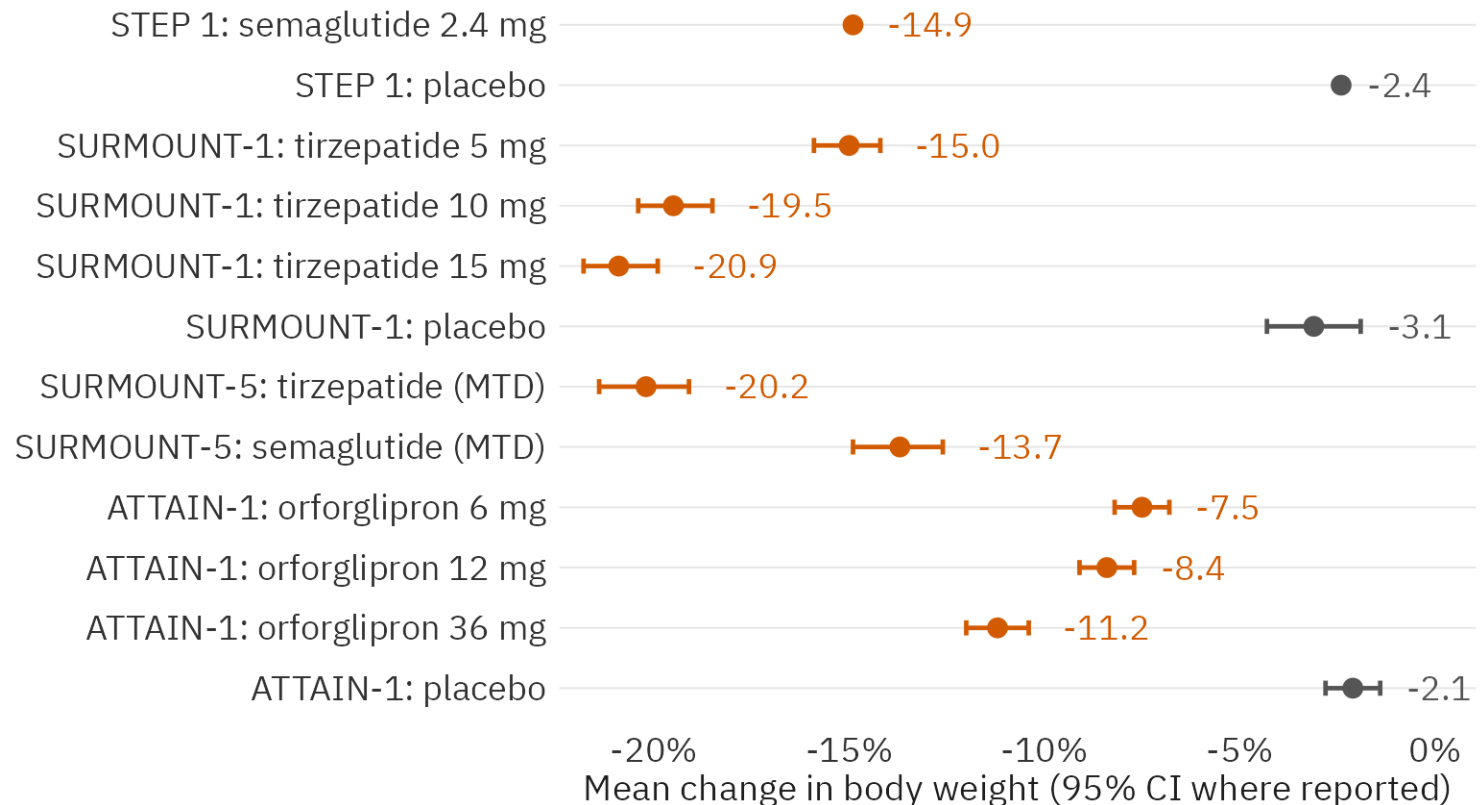
net per enrollee per month at the central rebate

Gross: \$5.51 per enrollee per month

\$1.14 at the announced \$245 price

Source: Module E. The rebate step is an assumption; the announced price is a program price, not a confirmed Medicaid net price.

Trials show average weight loss of 7.5% to 20.9%, set beside a Medicaid net cost of \$2,511 per user per year (context only)



\$2,511

net cost per user per year (4.3 fills, MEPS); \$6,943 per member-year of continuous treatment

Different populations, durations and doses

No cost-effectiveness claim is made

Trials: published abstracts via PubMed (STEP 1, SURMOUNT-1, SURMOUNT-5, ATTAIN-1); MTD = maximum tolerated dose (SURMOUNT-5). Costs: Module E, midpoint rebate, blended Wegovy and Zepbound.

Recommendations: considerations for a brand or access lead, from the evidence above

Finding	So what	What a brand or access lead might consider
An estimated 12.2 more prescriptions per 1,000 enrollees per quarter, growing over eight quarters	→ Uptake builds over about two years, not at launch	→ Plan access support for a multi-year ramp, not a launch spike
Coverage runs through managed care in South Carolina and Rhode Island	→ Managed-care organizations decide, as well as state programs	→ Aim evidence at managed-care medical and pharmacy directors
Coverage ended in California and Pennsylvania (-67.2% and -90.8%, preliminary)	→ Budget predictability decides whether coverage lasts	→ Show a cost range: \$2.69 per member per month centrally, \$1.14 at the announced price
The rebate is unknown and drives the range (\$55.6 to \$369.4 million)	→ Public data cannot settle net cost	→ Check the range against a payer's own net price

Considerations, not recommendations of a specific action; prescriber findings are in the report. Public aggregate data; no company affiliation or endorsement.

Appendix: data, methods, limits and links

Data. CMS Medicaid State Drug Utilization Data and enrollment, Medicare Part D Prescribers, NADAC, NHANES, MEPS, KFF polls, FDA labels and state Medicaid documents; all public aggregate data. 54 dbt models, 126 tests.

Methods. Callaway and Sant’Anna difference-in-differences (staggered adoption) with imputation of suppressed cells; survey-weighted NHANES estimates; ISPOR-style budget impact model with a probabilistic sensitivity analysis.

Limits. Gross, not net, of rebates; ten covering states; eligibility is a lower bound; Part D is not obesity use; scenarios, not forecasts.

Links. Interactive budget model: <https://erickyegon.github.io/incretin-access-value/budget-model.html> · Report: <https://erickyegon.github.io/incretin-access-value/report.html> · Site: <https://erickyegon.github.io/incretin-access-value/> · Key numbers: https://erickyegon.github.io/incretin-access-value/data/key_numbers.csv · Code: <https://github.com/erickyegon/incretin-access-value>

No company affiliation or endorsement. I used AI tools to help write code and documentation. The study design, methods and conclusions are my own, and I verified all results.